

ADDITIONAL CONTRACTING QUESTIONS & ANSWERS

#	Document	Page	Question	Answer
1	36C10G26R0015 NTP PHYSICAN SERVICES		Is this contract a recompetete or a new requirement solicitation?	This is a new requirement.
2	36C10G26R0015 NTP PHYSICAN SERVICES		This contraqct will require a significant investment in hardware and software to meet VA required IT standards and security. Will the USG add an advance payment clause to allow the awardee to purchase IT infrastruce required by VA regulations?	No, the government will not add an advanced payment clause.
3	NA	NA	Is there a current incumbent contractor? If yes, please provide the contractor's name and contract details.	No, there is no incumbent.
4	NA	NA	We respectfully request an extension of the proposal submission deadline by seven (7) business days following	The government will not extend the submission deadline.
5	NA	NA	Can you confirm whether this is intended to be a single-award or multiple-award IDIQ?	Please review RFP page 64 basis for award first sentance.
6	NA	NA	Is there a current incumbent supporting this requirement, and if so, is the incumbent eligible to compete?	No, there is no incumbent.
7	NA	NA	Is there any minimum volume guarantee beyond the stated minimum CLIN amount?	No.

ADDITIONAL TECHNICAL QUESTIONS & ANSWERS																																																																																																																																								
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1	Attachment A, PWS	28	Order limitations C4.b.1	N/A	Will the VA pay per unit per imaging modality regardless of volume?	VA will pay based on actual volume sent per unit per imaging modality quoted within the the maximum volume estimate. There is not																																																																																																																																		
2	Attachment A, PWS	10	5.1.2 DIRECT PATIENT CARE – TELERADIOLOGY	Deliverable	How many STAT imaging studies are in a 24 hour period?	This volume fluctuates. Historically approximately 75% of all studies read by NTP are STAT.																																																																																																																																		
3	Attachment A, PWS	10	5.1.2 DIRECT PATIENT CARE – TELERADIOLOGY	Deliverable	For a weekday, what is the breakdown by modality (XR, CT, MRI, US) and volume (number of studies) of work per hour for a 24 hour period? Within each hour and modality, how many of these reads are "STAT"?	This is historical data, to be used as reference only. <table><tr><th colspan="5">Weekdays</th></tr><tr><th>Weekday Hour</th><th>Avg/Day/Hour XR (%STAT)</th><th>Avg/Day/Hour CT (%STAT)</th><th>Avg/Day/Hour MR (%STAT)</th><th>Avg/Day/Hour US (%STAT)</th></tr><tr><td>0</td><td>29.8 (99.6%)</td><td>32.67 (97.6%)</td><td>1.67 (100.0%)</td><td>2.47 (100.0%)</td></tr><tr><td>1</td><td>25.8 (99.7%)</td><td>25.87 (98.7%)</td><td>0.73 (100.0%)</td><td>1.73 (100.0%)</td></tr><tr><td>2</td><td>28.6 (99.3%)</td><td>19.87 (95.6%)</td><td>1.73 (100.0%)</td><td>1.4 (100.0%)</td></tr><tr><td>3</td><td>34.6 (96.7%)</td><td>19.87 (94.6%)</td><td>1.33 (80.0%)</td><td>1.13 (100.0%)</td></tr><tr><td>4</td><td>33.33 (83.6%)</td><td>17.53 (79.5%)</td><td>2.53 (47.4%)</td><td>1.07 (93.8%)</td></tr><tr><td>5</td><td>23.93 (43.7%)</td><td>14.87 (65.9%)</td><td>4.07 (24.6%)</td><td>0.93 (57.1%)</td></tr><tr><td>6</td><td>39.07 (26.6%)</td><td>15.07 (52.7%)</td><td>5.53 (16.9%)</td><td>1.87 (28.6%)</td></tr><tr><td>7</td><td>28.13 (42.4%)</td><td>22.27 (42.8%)</td><td>6.4 (15.6%)</td><td>3.2 (54.2%)</td></tr><tr><td>8</td><td>34.4 (38.6%)</td><td>24.47 (40.1%)</td><td>8.13 (21.3%)</td><td>4.4 (42.4%)</td></tr><tr><td>9</td><td>46.67 (42.9%)</td><td>29.07 (45.4%)</td><td>7.4 (20.7%)</td><td>5.2 (37.2%)</td></tr><tr><td>10</td><td>41.53 (53.1%)</td><td>39.87 (39.5%)</td><td>9.27 (20.9%)</td><td>4.87 (41.1%)</td></tr><tr><td>11</td><td>57 (42.0%)</td><td>37.13 (42.4%)</td><td>10.87 (23.3%)</td><td>3.87 (41.4%)</td></tr><tr><td>12</td><td>60.87 (45.9%)</td><td>43.93 (47.0%)</td><td>12.67 (19.5%)</td><td>7.07 (43.4%)</td></tr><tr><td>13</td><td>111.53 (60.1%)</td><td>74.47 (76.6%)</td><td>20.4 (38.2%)</td><td>11.4 (72.5%)</td></tr><tr><td>14</td><td>150.87 (80.6%)</td><td>112.67 (86.1%)</td><td>17.6 (65.9%)</td><td>14.47 (86.2%)</td></tr><tr><td>15</td><td>150.67 (88.3%)</td><td>107.47 (93.6%)</td><td>18.4 (66.3%)</td><td>14.47 (86.6%)</td></tr><tr><td>16</td><td>129.6 (90.2%)</td><td>108.8 (93.9%)</td><td>19.53 (87.7%)</td><td>14 (96.2%)</td></tr><tr><td>17</td><td>114 (98.1%)</td><td>100.07 (98.1%)</td><td>11.27 (89.9%)</td><td>12.4 (99.5%)</td></tr><tr><td>18</td><td>104.07 (99.2%)</td><td>90.87 (98.2%)</td><td>10.47 (89.8%)</td><td>12.73 (95.3%)</td></tr><tr><td>19</td><td>86.47 (98.3%)</td><td>83.53 (99.1%)</td><td>9.2 (99.3%)</td><td>9.4 (99.3%)</td></tr><tr><td>20</td><td>72.73 (95.3%)</td><td>76.6 (97.9%)</td><td>7.4 (96.4%)</td><td>7.27 (99.1%)</td></tr><tr><td>21</td><td>54.4 (99.9%)</td><td>62.27 (96.9%)</td><td>5.33 (80.0%)</td><td>7.4 (100.0%)</td></tr><tr><td>22</td><td>38.6 (100.0%)</td><td>48.4 (98.6%)</td><td>1.73 (88.5%)</td><td>4.33 (100.0%)</td></tr><tr><td>23</td><td>40.8 (99.0%)</td><td>37.6 (98.1%)</td><td>1.67 (100.0%)</td><td>4.07 (100.0%)</td></tr></table>	Weekdays					Weekday Hour	Avg/Day/Hour XR (%STAT)	Avg/Day/Hour CT (%STAT)	Avg/Day/Hour MR (%STAT)	Avg/Day/Hour US (%STAT)	0	29.8 (99.6%)	32.67 (97.6%)	1.67 (100.0%)	2.47 (100.0%)	1	25.8 (99.7%)	25.87 (98.7%)	0.73 (100.0%)	1.73 (100.0%)	2	28.6 (99.3%)	19.87 (95.6%)	1.73 (100.0%)	1.4 (100.0%)	3	34.6 (96.7%)	19.87 (94.6%)	1.33 (80.0%)	1.13 (100.0%)	4	33.33 (83.6%)	17.53 (79.5%)	2.53 (47.4%)	1.07 (93.8%)	5	23.93 (43.7%)	14.87 (65.9%)	4.07 (24.6%)	0.93 (57.1%)	6	39.07 (26.6%)	15.07 (52.7%)	5.53 (16.9%)	1.87 (28.6%)	7	28.13 (42.4%)	22.27 (42.8%)	6.4 (15.6%)	3.2 (54.2%)	8	34.4 (38.6%)	24.47 (40.1%)	8.13 (21.3%)	4.4 (42.4%)	9	46.67 (42.9%)	29.07 (45.4%)	7.4 (20.7%)	5.2 (37.2%)	10	41.53 (53.1%)	39.87 (39.5%)	9.27 (20.9%)	4.87 (41.1%)	11	57 (42.0%)	37.13 (42.4%)	10.87 (23.3%)	3.87 (41.4%)	12	60.87 (45.9%)	43.93 (47.0%)	12.67 (19.5%)	7.07 (43.4%)	13	111.53 (60.1%)	74.47 (76.6%)	20.4 (38.2%)	11.4 (72.5%)	14	150.87 (80.6%)	112.67 (86.1%)	17.6 (65.9%)	14.47 (86.2%)	15	150.67 (88.3%)	107.47 (93.6%)	18.4 (66.3%)	14.47 (86.6%)	16	129.6 (90.2%)	108.8 (93.9%)	19.53 (87.7%)	14 (96.2%)	17	114 (98.1%)	100.07 (98.1%)	11.27 (89.9%)	12.4 (99.5%)	18	104.07 (99.2%)	90.87 (98.2%)	10.47 (89.8%)	12.73 (95.3%)	19	86.47 (98.3%)	83.53 (99.1%)	9.2 (99.3%)	9.4 (99.3%)	20	72.73 (95.3%)	76.6 (97.9%)	7.4 (96.4%)	7.27 (99.1%)	21	54.4 (99.9%)	62.27 (96.9%)	5.33 (80.0%)	7.4 (100.0%)	22	38.6 (100.0%)	48.4 (98.6%)	1.73 (88.5%)	4.33 (100.0%)	23	40.8 (99.0%)	37.6 (98.1%)	1.67 (100.0%)	4.07 (100.0%)
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16	95.67 (93.7%)	76.67 (88.3%)	6.83 (53.7%)	11.33 (91.2%)																																																																																																																																				
17	83.83 (98.4%)	75 (98.0%)	5 (73.3%)	6.83 (87.8%)																																																																																																																																				
18	73.83 (99.6%)	58.17 (95.4%)	7.67 (69.6%)	5.83 (91.4%)																																																																																																																																				
19	63.33 (100.0%)	64.67 (98.7%)	2.83 (64.7%)	4.17 (96.0%)																																																																																																																																				
20	43.33 (99.6%)	62.5 (93.9%)	0.83 (100.0%)	4.17 (96.0%)																																																																																																																																				
21	35.17 (100.0%)	34.33 (94.7%)	2.67 (87.5%)	3 (94.4%)																																																																																																																																				
22	40.83 (99.6%)	36.67 (95.9%)	1.67 (80.0%)	3.5 (95.2%)																																																																																																																																				
23	35.33 (100.0%)	40.17 (99.6%)	0.67 (100.0%)	4 (100.0%)																																																																																																																																				
5	Attachment A, PWS	10	5.1.2 DIRECT PATIENT CARE – TELERADIOLOGY	Deliverable	For CT imaging, what is the volume and percentage breakdown by the following: CT head, CT neck, CT chest, CT abdomen/pelvis, CT cervical spine, CT thoracic spine, CT lumbar spine, CT musculoskeletal, CT other (please list others)?	This is historical data, to be used as reference only. <table><thead><tr><th>CT Category</th><th>CPT codes included</th><th>NTP 2025 Volume</th><th>%</th></tr></thead><tbody><tr><td>CT Head</td><td>70450, 70460, 70470</td><td>113922</td><td>21.82%</td></tr><tr><td>CT neck</td><td>70480, 70481, 70482</td><td>2639</td><td>0.51%</td></tr><tr><td>CT Chest</td><td>71250, 71260, 71270</td><td>63259</td><td>12.11%</td></tr><tr><td>CT Abd/Pel</td><td>74176, 74177, 74178</td><td>146212</td><td>28.00%</td></tr><tr><td>CT Cervical Spine</td><td>72125, 72126, 72127</td><td>30825</td><td>5.90%</td></tr><tr><td>CT Thoracic Spine</td><td>72128, 72129, 72130</td><td>5115</td><td>0.98%</td></tr><tr><td>CT Lumbar Spine</td><td>72131, 72132, 72133</td><td>14278</td><td>2.73%</td></tr><tr><td>CT Musculoskeletal</td><td>73200, 73201, 73202, 73700, 73701, 73702</td><td>14066</td><td>2.69%</td></tr><tr><td>CT Other</td><td>All other CT Codes not listed above</td><td>131872</td><td>25.25%</td></tr></tbody></table>	CT Category	CPT codes included	NTP 2025 Volume	%	CT Head	70450, 70460, 70470	113922	21.82%	CT neck	70480, 70481, 70482	2639	0.51%	CT Chest	71250, 71260, 71270	63259	12.11%	CT Abd/Pel	74176, 74177, 74178	146212	28.00%	CT Cervical Spine	72125, 72126, 72127	30825	5.90%	CT Thoracic Spine	72128, 72129, 72130	5115	0.98%	CT Lumbar Spine	72131, 72132, 72133	14278	2.73%	CT Musculoskeletal	73200, 73201, 73202, 73700, 73701, 73702	14066	2.69%	CT Other	All other CT Codes not listed above	131872	25.25%																																																																																										
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6	Attachment A, PWS	10	5.1.2 DIRECT PATIENT CARE – TELERADIOLOGY	Deliverable	For MR imaging, what is the volume and percentage breakdown by the following: MRI brain, MRI cervical spine, MRI thoracic spine, MRI lumbar spine, MRI abdomen, MRI pelvis, MRA head, MRA neck, MRI other (please list others)?	This is historical data, to be used as reference only. <table><thead><tr><th>MRI Category</th><th>CPT codes included</th><th>NTP 2025 Volume</th><th>%</th></tr></thead><tbody><tr><td>MRI brain</td><td>70551, 70553</td><td>19553</td><td>23.70%</td></tr><tr><td>MRI cervical spine</td><td>72141, 72156</td><td>9022</td><td>10.94%</td></tr><tr><td>MRI thoracic spine</td><td>72146, 72157</td><td>3822</td><td>4.63%</td></tr><tr><td>MRI lumbar spine</td><td>72148, 72158</td><td>13757</td><td>16.68%</td></tr><tr><td>MRI abdomen</td><td>74181, 74183</td><td>8661</td><td>10.50%</td></tr><tr><td>MRI pelvis</td><td>72195, 72197</td><td>2403</td><td>2.91%</td></tr><tr><td>MRA head</td><td>70544, 70545</td><td>3035</td><td>3.68%</td></tr><tr><td>MRA neck</td><td>70547, 70548</td><td>1594</td><td>1.93%</td></tr><tr><td>MRI other</td><td>All other MR Codes not listed above</td><td>20643</td><td>25.02%</td></tr></tbody></table>	MRI Category	CPT codes included	NTP 2025 Volume	%	MRI brain	70551, 70553	19553	23.70%	MRI cervical spine	72141, 72156	9022	10.94%	MRI thoracic spine	72146, 72157	3822	4.63%	MRI lumbar spine	72148, 72158	13757	16.68%	MRI abdomen	74181, 74183	8661	10.50%	MRI pelvis	72195, 72197	2403	2.91%	MRA head	70544, 70545	3035	3.68%	MRA neck	70547, 70548	1594	1.93%	MRI other	All other MR Codes not listed above	20643	25.02%																																																																																										
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7	Attachment A, PWS	17	5.1.4 CONTINUITY OF SERVICES UNDER CONTRACT	5. Integration with VHA PACS and EMR's	When performing HL7 integration with the VA, is there a single mechanism for this integration to encompass all associated approximately 130 VA facilities, or does this integration have to be performed separately at each VA site?	VA VistA sites use either Radiology HL7 2.3 or HL7 2.4 while Oracle Health sites use a single EMR and interface. Each VistA site will require separate integration/validation. Each Oracle Health site will similarly require integration/validation even though it is a single interface.
8	Attachment A, PWS	17	5.1.4 CONTINUITY OF SERVICES UNDER CONTRACT	5. Integration with VHA PACS and EMR's	How long does the VA estimate the HL7 integration will take?	VA utilizes standard ORM and ORU interfaces for VistA HL7 v2.3 and 2.4 and for Oracle Health Federal Electronic Health Record. The vendor will be required to conform to VA's interface specifications. Depending on vendor's familiarity with VA specifications, it may take between 90 and 120 days to create and validate the interfaces.
9	Attachment A, PWS	33	4. INFORMATION SYSTEM DESIGN AND DEVELOPMENT	j	How many days to remediate?	Two days, please see updated PWS, but also referenced on page 15 of the solicitation.
10	Attachment A, PWS	34	4. INFORMATION SYSTEM DESIGN AND DEVELOPMENT	k	How many days does the vendor have to apply the security fixes?	Two days, please see updated PWS, but also referenced on page 15 of the solicitation
11	Attachment A, PWS	39	7. LIQUIDATED DAMAGES FOR DATA BREACH	d	What is the liquidated damages amount?	\$37.50, please see updated PWS, but also referenced on page 19 of the solicitation.
12	Attachment A, PWS	N/A	N/A	N/A	What are the states/locations that will require teleradiology coverage?	This is a nation wide contract. The NTP provides teleradiology services to more than 130 VHA facilities across the United States. Please review page 3 of PWS.
13	Attachment A, PWS	15-16	5.1.3 CONTRACTOR RESPONSIBILITIES	16. No Employee status:	What are the minimum requirements for "Workers' compensation" and "Professional liability insurance" required for a VA contractor?	Please review solicitation page 43 supplemental insurance for "workers compensation"
14	Attachment A, PWS	9	5.1.2 DIRECT PATIENT CARE – TELERADIOLOGY	"The Contract Radiologist(s) will immediately notify VA clinicians of critical findings in accordance with American College of Radiology (ACR) standards, VHA and the NTP "alert notification" Quality Assurance policies, and in a manner appropriate to the clinical urgency of	Regarding discussion of emergent/critical findings from imaging studies or other findings that could necessitate communication with the radiologist and ordering provider, how will the VA provide the appropriate contact information for the ordering provider?	Requesting provider information is contained within the HL7 order message. However, best practice is to develop a site by site contact list for ERs, ICUs etc. in case the contact information from the EMR information is out of date.
15	Solicitation	60			On page #60 under Factor 2, the solicitation states that the system can be adjusted as needed by the offeror's personnel to meet workflow management needs. Can you please clarify this requirement and provide specific examples of what types of system adjustments are expected?	This is intended to solicit a response on how offeror can adjust their workflow in response to variable workload. Should be specific to your internal processes and/or systems.
16	Solicitation	60			VA OIT no longer issues Memoranda of Understanding (MOU) between companies and VHA until after award. We respectfully request that this requirement be removed. If it is not removed, we request that the Contracting Officer provide assistance to offerors in obtaining an MOU with VA OIT.	Please review amendment 0001 page 6, for updated instruction to offerors. Instructions will be provided to the awardee.
17	Solicitation	60			We also request that the Contracting Officer assist offerors in obtaining an Interconnection Security Agreement (ISA), as these agreements cannot be provided unless specifically initiated by the Government.	Please review Amendment 0001 page 6, for updated instruction to offerors. Instructions will be provided to the awardee.

18	PWS	10			The solicitation indicates approximately 51,000 CT scans annually, which equates to more than 140 scans per day. This appears disproportionate compared to typical imaging distributions where X-rays significantly exceed CT scans. Can you please confirm whether this estimate is accurate?	Yes it is accurate.
19	PWS	10			Can you provide a percent breakdown of CT scans and X-rays by time period, specifically the percentage performed during daytime hours (8a-5p) versus nighttime (5p-8a) and weekends?	See answer in row 5 and 6. Hours is in Pacific Time.
20					Can you confirm whether the estimated study volumes provided represent historical actuals or projected demand?	Projected based on historical data and growth trends.
21					Are there any known challenges or performance gaps with the current or prior solution that offerors should be aware of to ensure continuity and quality of care	No.

ADDITIONAL PAST PERFORMANCE QUESTIONS & ANSWERS						
#	Document	Page	Section	Paragraph (if applicable)	Question	Government Answer
1			(ii) VOLUME II – PAST PERFORMANCE FACTOR	<i>Satisfactory Performance</i>	A neutral rating is normally applied if a company does not have direct past performance in a specific area. Will the USG allow a neutral rating.	Please review pages 66-67 of the RFP.
2			(ii) VOLUME II – PAST PERFORMANCE FACTOR	<i>Satisfactory Performance</i>	Will individual Past Performance by proposed physicians be accepted?	The government will evaluate in accordance with the past performance evaluation approach, please review pages 66-67 of the RFP.
3	Solicitation	61	VOLUME II – PAST PERFORMANCE FACTOR	NA	On page #61 regarding relevancy, we have multiple contracts providing teleradiology services across various facilities and states. While no single contract has an annual value of \$6 million, the combined value of multiple contracts exceeds \$40 million a year or a single contract over 5 years exceeds the \$6 million a year. We request Government to change this requirement to \$6 million for the entire period of contract instead of annual value of contract.	The \$6m for annual dollar value remains unchanged.
4	Solicitation	61	VOLUME II – PAST PERFORMANCE FACTOR	NA	We have multiple Government past performance references with annual values less than \$6 million, as well as commercial contracts exceeding \$6 million annually. How will the Government evaluate commercial past performance in comparison to Government past performance? Will evaluation be based primarily on contract size?	It will be evaluated the same, the only difference is whether you have CPARS/PPIRS or PPQ. Please review page 61 of the RFP.